AC-12 Termination of Resuscitation

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use only for adult, nontraumatic out-of-hospital cardiac arrest after ongoing high-quality resuscitation. Medical Control authorization is required.

Adult nontraumatic termination-of-resuscitation rule

1. Begin/continue resuscitation	Follow AC-03/AC-11 and the rhythm pathway for at least 20 minutes of high-quality care, airway/ventilation, access/medications as indicated, and reversible-cause assessment.
2. Screen exclusions	Do not use this rule for pediatric, traumatic, hypothermic, poisoning/overdose, pregnancy, mechanical-device, or other special-circumstance arrests; use the linked pathway and Medical Control.
3. Universal TOR criteria	All are required: not witnessed by EMS; no shock delivered before transport; and no ROSC before transport.
4. All three present?	If no: continue resuscitation and consider transport. If yes: evaluate full clinical context and contact Medical Control.
5. Medical Control report	Report criteria/exclusions, care duration, rhythms, shocks, medications, airway/capnography/EtCO2 trend, reversible causes, and any special factors.
6. Authorization	If authorized: stop CPR, document time, support family, preserve scene, and follow disposition policy. If not authorized/unavailable: continue resuscitation and transport.

Key treatment and handoff

Item	Approved action
DNR/MOST/obvious death	Honor valid directives and follow the separate obvious-death/death-in-field policy; this is not a substitute for that policy.
Family	Use clear, compassionate communication and follow agency death-notification/bereavement procedures.

No single EtCO2 value, rhythm, downtime estimate, or absence of a pulse alone authorizes termination.

AC-12 Termination of Resuscitation

SAFETY, LINKED PATHWAYS, AND DOCUMENTATION

Safety checks

- Continue full care while eligibility is evaluated and Medical Control is contacted.

- Document every criterion, exclusion screen, Medical Control physician/order, time of termination, notifications, family support, and disposition.

Linked pathways

Pathway	Use when
AC-03	Adult cardiac arrest
AC-11	TEAM-focused CPR
TE-03/TE-05/TE-07	Drowning, hypothermia, or toxicologic special circumstances

Documentation and release control

- Document the presenting condition, focused findings, rhythm/ECG when applicable, all interventions with dose/route/time/response, serial vital signs, reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.